

NovoCure

NASDAQ : \$1.86B mkt cap · 109M sh · \$1.03B cash, \$560M debt · \$655M revenue FY25 (+8%, ~78% GM); GAAP net loss -\$136M, adj-EBITDA -\$34M; NVCR · ~\$0.47B NET CASH (p_fail~0); 4 FDA-approved/filed indications: Optune Gio (GBM), Lua (NSCLC, early), Pax (pancreatic), METIS (brain mets); FRESH NEGATIVE: Jun-18-2026 Phase-3 TRIDENT (newly-dx GBM) MISSED → -18%; SBC ~\$100M+/yr Company DCF

\$17.10

THE CENTRAL QUESTION

NovoCure trades at ~2.5x sales with net cash and an unproven new-indication tail — does the optionality beyond glioblastoma justify the price, or does it stay a loss-making device story?

NovoCure sells Tumor Treating Fields (TTFields), a wearable oncology device anchored by Optune Gio in glioblastoma. FY2025 revenue was \$655M (+8%) at a 78% gross margin, yet it posted a \$136M GAAP loss — funded by ~\$0.47B net cash, not insolvency risk. At \$14.33 the stock is cheap on sales; the upside lives in new FDA-approved indications (Optune Lua/lung, Pax/pancreatic, METIS) multiplying the TAM beyond GBM. Weighted fair value \$14.97, ~4.5% above spot — fair, with the June 2026 TRIDENT miss capping the GBM leg.

PROBABILITY-WEIGHTED EXPECTED VALUE

\$14.97 expected fair value today
-12.4% vs spot \$17.10

FORWARD COMPOUNDED VALUE

+5y	+10y	+15y	+20y
\$23.34	\$36.41	\$56.83	\$88.76
1.37× spot	2.13× spot	3.32× spot	5.19× spot

WEIGHTING RATIONALE

Ultra Bear 14% GBM erodes post-TRIDENT; new indications never adopt; \$4.20.

Bear 28% Slow adoption, thin leverage; small owner-FCF; \$9.19.

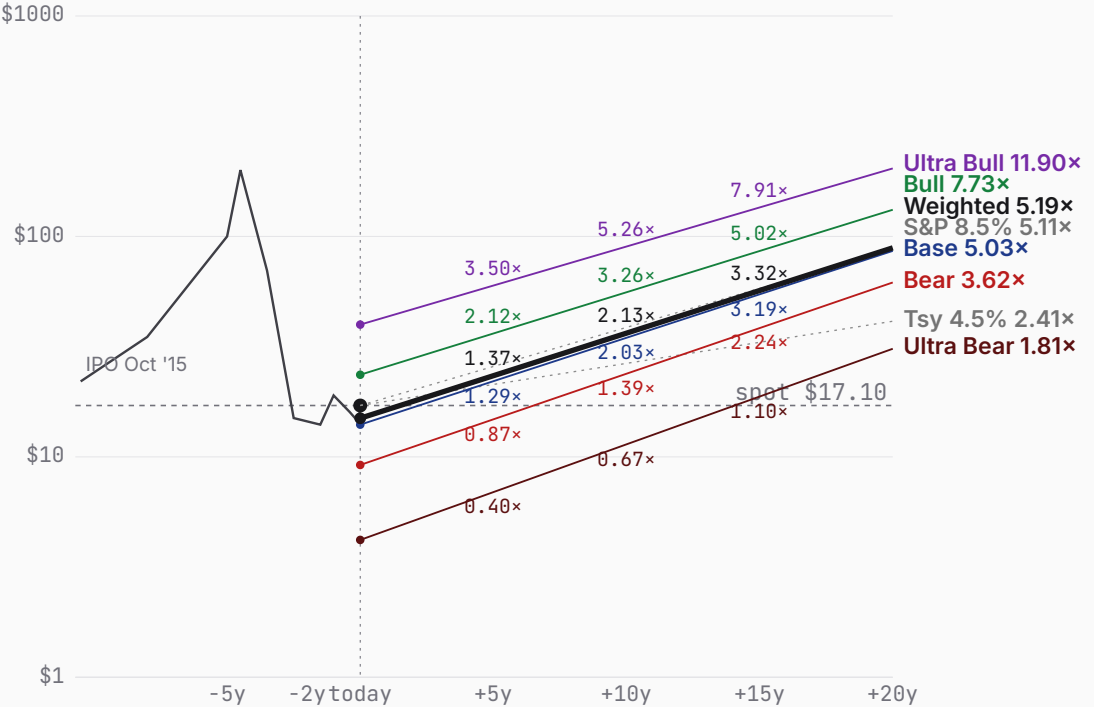
Base 33% Moderate new-indication ramp; margin ~10%; ~fair, \$14.00.

Bull 17% Lua and Pax adopt, the 78% margin levers to 14%; \$23.58.

Ultra Bull 9% TTFields a multi-indication standard; \$39.81.

Bull (17%) + Ultra Bull (8%) together contribute \$7.19 — 48% of the \$14.97 expected value despite 25% combined probability. Today's spot (\$17.10) sits 14% above the weighted expected; forward-weighted value crosses spot between +5y and +10y.

PRICE + PROJECTIONS · HISTORICAL THROUGH PROBABILITY-WEIGHTED FORWARD



ULTRA BEAR	\$4.20	BEAR	\$9.19	BASE	\$14.00	BULL	\$23.58	ULTRA BULL	\$39.81
	-75.4% vs spot		-46.3% vs spot		-18.1% vs spot		+37.9% vs spot		+132.8% vs spot
CAGR +1.8% WACC 10.5% CAGR +1.8% Prob 14%		CAGR +4.0% WACC 10.0% CAGR +4.0% Prob 28%		CAGR +5.6% WACC 9.5% CAGR +5.6% Prob 33%		CAGR +8.2% WACC 9.0% CAGR +8.2% Prob 17%		CAGR +10.8% WACC 8.5% CAGR +10.8% Prob 8%	
GBM erodes; new indications fail		Slow adoption, thin operating leverage		New indications ramp; margin inflects		Lua/Pax adopt; the margin levers		TTFields a multi-indication standard	



NovoCure scenario narratives

PROBABILITY WEIGHTS

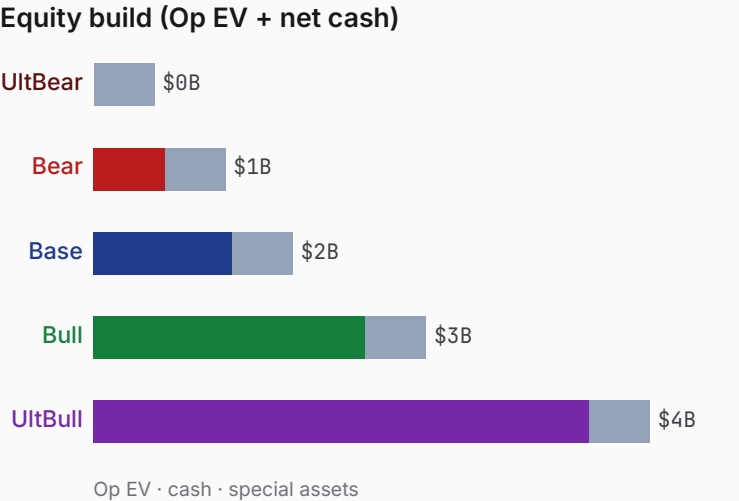
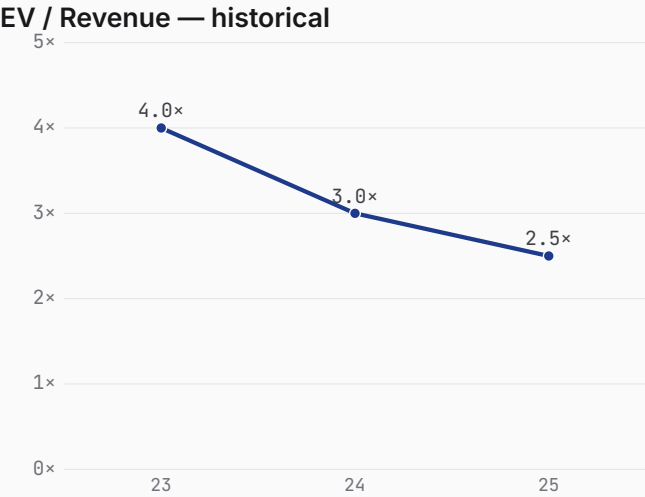
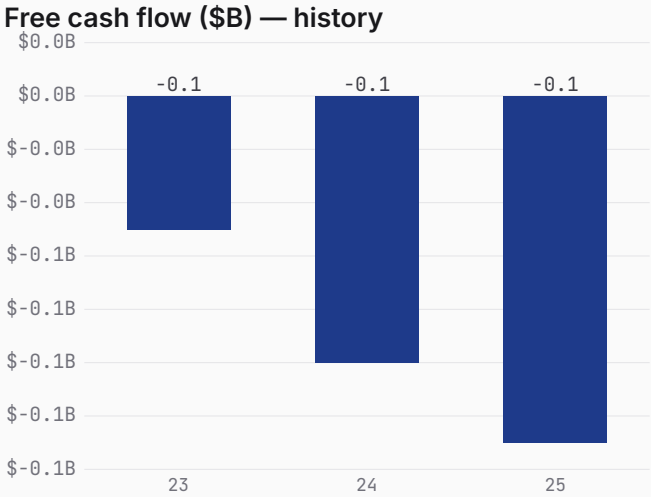
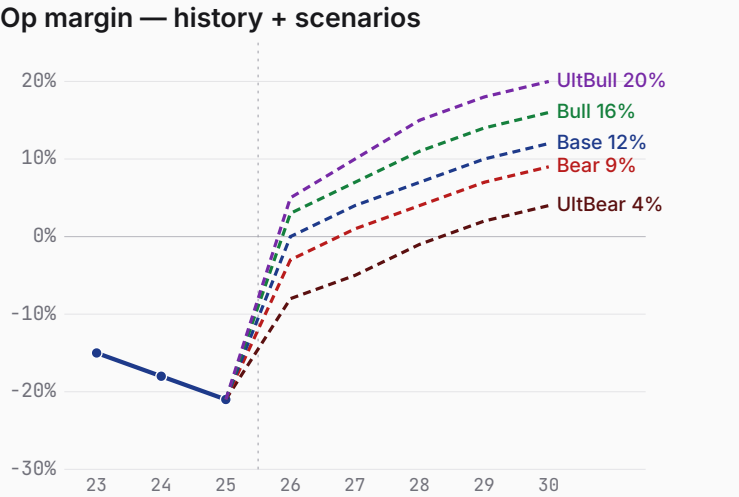
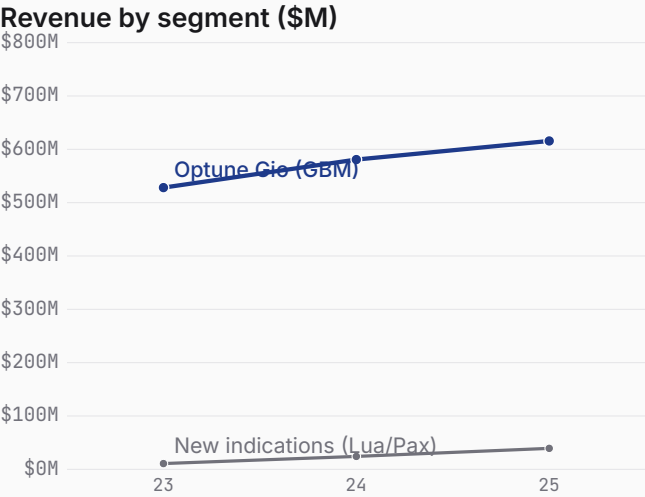
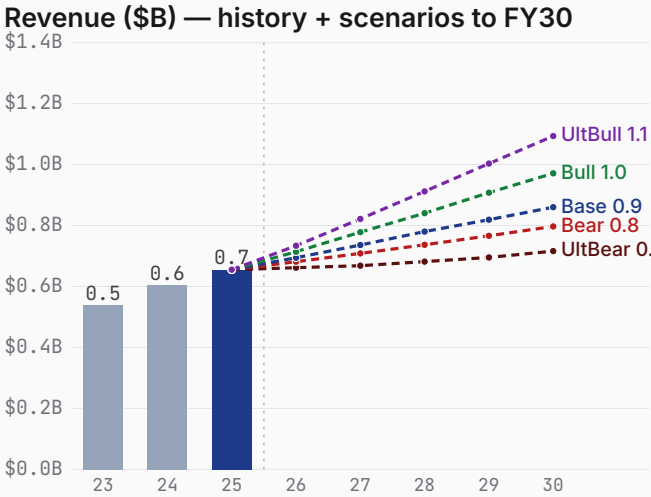
Ultra Bear 14% Bear 28% Base 33% Bull 17% Ultra Bull 8% · Weighted expected \$14.97 (-12.4% vs spot)

ULTRA BEAR	\$4.20	BEAR	\$9.19	BASE	\$14.00	BULL	\$23.58	ULTRA BULL	\$39.81
	-75.4% vs spot		-46.3% vs spot		-18.1% vs spot		+37.9% vs spot		+132.8% vs spot
Probability 14%		Probability 28%		Probability 33%		Probability 17%		Probability 8%	
GBM erodes; new indications fail		Slow adoption, thin operating leverage		New indications ramp; margin inflects		Lua/Pax adopt; the margin levers		TTFields a multi-indication standard	
WHY 14%		WHY 28%		WHY 33%		WHY 17%		WHY 8%	
Roughly one-in-seven. Requires both the GBM franchise softening after TRIDENT and the new indications stalling, while the gross margin never translates into operating profit — plausible given the long loss history, but it ignores three already-approved labels.		About 28%. The modal disappointment path — labels exist but real-world wear, reimbursement gating, and heavy stock comp blunt the margin inflection. Consistent with a company that has grown revenue but never produced operating profit.		The plurality outcome at ~33%. New indications adopt at a moderate, not heroic, rate and the long-promised operating leverage finally shows up around 10% — neither the post-TRIDENT erosion nor a platform breakout, just steady execution on approved labels.		About 17%. Needs genuine commercial traction in the new indications plus the first durable operating profit in the company's history — both observable in active-patient counts and segment revenue, neither yet demonstrated at scale.		About 9%. The full platform thesis — TTFields as a multi-tumor standard of care with proven leverage. The low weight reflects unproven leverage and the fresh TRIDENT miss, not the absence of a plausible mechanism.	
WHAT HAPPENS		WHAT HAPPENS		WHAT HAPPENS		WHAT HAPPENS		WHAT HAPPENS	
Revenue grows only ~1.8% as the post-TRIDENT GBM base flattens and the newer labels fail to convert into real prescriptions. Active patients stall near today's ~4,800, and continuous-wear compliance keeps real-world uptake well below the approved population.		Device adoption stays sluggish: Optune Lua is still only ~\$3M a quarter, and Pax ramps slowly against entrenched chemo and radiation pathways. Revenue compounds ~4% as new indications add patients but oncologist and patient inertia caps the pace.		Revenue compounds ~5.6% as Optune Lua and Pax build a modest second and third leg on top of GBM, and METIS adds an NSCLC-brain-mets indication after its PMA filing. Active-patient growth continues at roughly the current single-digit pace across a widening label set.		Optune Lua and Pax adopt well in their approved settings, and METIS converts, so revenue compounds ~8.2%. Active patients climb materially as TTFields becomes a routine option in multiple tumor types rather than a GBM-only therapy, with reimbursement coverage broadening alongside the labels.		TTFields establishes itself as a standard modality across several solid tumors, with positive trials extending the platform beyond today's labels. Revenue compounds ~10.8% as the addressable population multiplies and continuous-wear devices become a routine part of oncology protocols.	
Owner-FCF margin starts at -10% and crawls to 2% — the operating leverage never arrives after a decade of losses. At ~1.8% growth and a barely-positive terminal margin the DCF is \$4.20, ~71% below spot. Net cash prevents distress, but the equity is worth little if TTFields stays a single-indication niche.		The owner-FCF margin moves from -5% to 7% — positive eventually, but thin, because SBC over \$100M a year keeps true owner cash well below the adj-EBITDA headline. The DCF lands at \$9.19, ~36% below spot: a real, funded business that simply doesn't earn enough to support the current price.		The owner-FCF margin inflects from -2% toward 10% as the 78% gross margin starts to cover a more fixed cost base. That yields a \$14.00 DCF, ~2% below spot — essentially fair value. The market is paying for moderate multi-indication adoption and a believable, if late, path to profitability.		Operating leverage finally arrives: the owner-FCF margin runs from 1% to 14% as the high gross margin drops through a stable cost base. The DCF is \$23.58, ~65% above spot. This requires the device-adoption gap (labels outrunning real prescriptions) to close, which it has not yet across the newer indications.		The owner-FCF margin scales from 3% to 18% as a unique, patented modality earns device-like economics on a much larger base. The DCF reaches \$39.81, ~178% above spot. This is the platform outcome: the IP and clinical-evidence moat compounding across indications rather than defending a single one.	

NovoCure business snapshot

Bear \$9.19 Base \$14.00 Bull \$23.58

FY23-FY25 history + FY26-FY30 scenario projections · fiscal years end Dec 31 · FY2025 + Q1'26 results



Sources: NovoCure FY2025 10-K + Q1'26 results (CIK 1645113), FY26 guidance, TRIDENT/PANOVA-3/METIS readouts. Revenue as a growth-rate path off FY25; FCF = revenue x an SBC-adjusted owner-FCF margin (starts negative — device operating leverage is unproven and SBC is ~\$100M+/yr); Gordon terminal at scenario WACC. EV/sales vs med-tech / oncology peers.

NovoCure 7 Powers · the moat, scored

Bear \$9.19 Base \$14.00 Bull \$23.58

Arena. Solid-tumor oncology, where a wearable TTFields device competes for oncologist and patient mindshare against systemic drugs and radiation, not against a rival device.

POWER AUDIT
dominant-power durability: medium

POWER AUDIT · 7 POWERS

Scale Economies 	Some fixed-cost leverage in R&D and field operations, but the model is still loss-making after years.
Network Economies 	No network effect; value to one patient does not rise with the number of other users.
Counter-Positioning 	A non-systemic modality incumbents (pharma, radiation) cannot easily add without cannibalizing or re-tooling around drugs.
Switching Costs 	Modest — an in-progress treatment course is sticky, but adoption is per-line and physician-driven, not locked-in.
Branding 	Optune has oncology recognition, but prescribing follows clinical evidence and reimbursement, not brand.
Cornered Resource · thesis leans here 	The TTFields IP and patented device platform are unique — no direct device competitor delivers this modality.
Process Power 	Accumulated regulatory and trial know-how across indications, but reproducible by a well-funded entrant over time.

PEER SET

Standard chemo / radiation The real competition for oncologist mindshare; entrenched, reimbursed, default pathway.	· n/a (standard of care)
Pharma oncology (IO / targeted) Systemic drugs competing for the same treatment line; vastly larger R&D budgets.	· large-cap pharma multiples
Other neuro-oncology device / trials Few direct device rivals today; the threat is future entrants replicating the modality.	· early-stage / venture
Build-vs-adopt inertia Continuous-wear compliance and physician habit, not a competitor, gate real-world uptake.	· n/a

THREAT VECTORS · FALSIFIERS

Cornered Resource · Future device entrants falsifier: A competing electric-field or wearable modality clearing trials and eroding the patent/platform exclusivity.
Counter-Positioning · Payers / reimbursement falsifier: Coverage restrictions or denials gating new indications, which keeps real prescriptions below approved populations.
Process Power · New-indication trial risk falsifier: A second pivotal miss after TRIDENT in lung, pancreatic, or brain mets — undercutting the clinical-evidence moat that justifies each new label.

COMPETITIVE READ

NovoCure has a genuine cornered resource — the TTFields IP and the only approved device for this modality — reinforced by a per-indication clinical-evidence moat (PANOVA-3, METIS). That makes the modality largely uncontested by direct device rivals. But the Power is medium-durability, not high: real-world adoption lags labels, reimbursement gates uptake, and the June 2026 TRIDENT miss shows the moat must be re-won trial by trial. The falsifier is a second pivotal miss or flat active-patient growth — either would prove the platform cannot extend beyond GBM.

NovoCure show your work

Bear \$9.19 Base \$14.00 Bull \$23.58 · Weighted \$14.97 (-12.4%)

ULTRA BEAR					\$4.20	BEAR					\$9.19	BASE					\$14.00	BULL					\$23.58	ULTRA BULL					\$39.81						
ASSUMPTIONS						ASSUMPTIONS						ASSUMPTIONS						ASSUMPTIONS						ASSUMPTIONS											
5y revenue CAGR (%)					+1.8	5y revenue CAGR (%)					+4.0	5y revenue CAGR (%)					+5.6	5y revenue CAGR (%)					+8.2	5y revenue CAGR (%)					+10.8						
Year-5 op margin (%)					4.0	Year-5 op margin (%)					9.0	Year-5 op margin (%)					12.0	Year-5 op margin (%)					16.0	Year-5 op margin (%)					20.0						
WACC (%)					10.5	WACC (%)					10.0	WACC (%)					9.5	WACC (%)					9.0	WACC (%)					8.5						
Terminal growth (%)					2.5	Terminal growth (%)					3.0	Terminal growth (%)					3.5	Terminal growth (%)					4.0	Terminal growth (%)					4.5						
Terminal FCF (\$B)					\$0.01B	Terminal FCF (\$B)					\$0.06B	Terminal FCF (\$B)					\$0.09B	Terminal FCF (\$B)					\$0.14B	Terminal FCF (\$B)					\$0.20B						
Starting revenue (\$B)					0.66	Starting revenue (\$B)					0.66	Starting revenue (\$B)					0.66	Starting revenue (\$B)					0.66	Starting revenue (\$B)					0.66						
Scenario probability (%)					14	Scenario probability (%)					28	Scenario probability (%)					33	Scenario probability (%)					17	Scenario probability (%)					8						
5-YEAR DCF · \$B						5-YEAR DCF · \$B						5-YEAR DCF · \$B						5-YEAR DCF · \$B						5-YEAR DCF · \$B											
FY	Rev	Margin	FCF	PV FCF		FY	Rev	Margin	FCF	PV FCF		FY	Rev	Margin	FCF	PV FCF		FY	Rev	Margin	FCF	PV FCF		FY	Rev	Margin	FCF	PV FCF		FY	Rev	Margin	FCF	PV FCF	
FY26	0.66	-8%	-0.07	-0.06		FY26	0.68	-3%	-0.03	-0.03		FY26	0.69	+0%	-0.01	-0.01		FY26	0.71	+3%	+0.01	+0.01		FY26	0.73	+5%	+0.02	+0.02		FY26	0.76	+8%	+0.04	+0.04	
FY27	0.67	-5%	-0.05	-0.04		FY27	0.71	+1%	-0.01	-0.01		FY27	0.74	+4%	+0.01	+0.01		FY27	0.78	+7%	+0.04	+0.03		FY27	0.82	+10%	+0.07	+0.06		FY27	0.86	+12%	+0.09	+0.08	
FY28	0.68	-1%	-0.02	-0.01		FY28	0.74	+4%	+0.01	+0.01		FY28	0.78	+7%	+0.04	+0.03		FY28	0.84	+11%	+0.08	+0.06		FY28	0.91	+14%	+0.11	+0.09		FY28	0.97	+16%	+0.14	+0.11	
FY29	0.70	+2%	+0.00	+0.00		FY29	0.77	+7%	+0.04	+0.03		FY29	0.82	+10%	+0.07	+0.05		FY29	0.91	+14%	+0.11	+0.08		FY29	1.00	+18%	+0.16	+0.12		FY29	1.09	+20%	+0.20	+0.16	
FY30	0.72	+4%	+0.01	+0.01		FY30	0.80	+9%	+0.06	+0.04		FY30	0.86	+12%	+0.09	+0.06		FY30	0.97	+16%	+0.14	+0.09		FY30	1.09	+20%	+0.20	+0.13		FY30	1.21	+24%	+0.26	+0.20	
Σ PV explicit FCF					-0.11	Σ PV explicit FCF					+0.04	Σ PV explicit FCF					+0.13	Σ PV explicit FCF					+0.26	Σ PV explicit FCF					+0.41						
+ PV terminal (g 2.5%)					0.11	+ PV terminal (g 3.0%)					0.51	+ PV terminal (g 3.5%)					0.94	+ PV terminal (g 4.0%)					1.84	+ PV terminal (g 4.5%)					3.42						
EQUITY BUILD · \$B & PER SHARE						EQUITY BUILD · \$B & PER SHARE						EQUITY BUILD · \$B & PER SHARE						EQUITY BUILD · \$B & PER SHARE						EQUITY BUILD · \$B & PER SHARE											
Operating EV					\$0.00B	Operating EV					\$0.55B	Operating EV					\$1.07B	Operating EV					\$2.10B	Operating EV					\$3.83B						
+ Cash & investments					\$1.03B	+ Cash & investments					\$1.03B	+ Cash & investments					\$1.03B	+ Cash & investments					\$1.03B	+ Cash & investments					\$1.03B						
– Total debt					\$0.56B	– Total debt					\$0.56B	– Total debt					\$0.56B	– Total debt					\$0.56B	– Total debt					\$0.56B						
= Equity value					\$0.47B	= Equity value					\$1.02B	= Equity value					\$1.54B	= Equity value					\$2.57B	= Equity value					\$4.30B						
FY30 shares (M)					112	FY30 shares (M)					111	FY30 shares (M)					110	FY30 shares (M)					109	FY30 shares (M)					108						
= Expected per share					\$4.20	= Expected per share					\$9.19	= Expected per share					\$14.00	= Expected per share					\$23.58	= Expected per share					\$39.81						
FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT						FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT						FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT						FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT						FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT											
+5y	+10y	+15y	+20y		+5y	+10y	+15y	+20y		+5y	+10y	+15y	+20y		+5y	+10y	+15y	+20y		+5y	+10y	+15y	+20y		+5y	+10y	+15y	+20y		+5y	+10y	+15y	+20y		
\$6.92	\$11.40	\$18.78	\$30.94		\$14.80	\$23.84	\$38.39	\$61.83		\$22.04	\$34.70	\$54.62	\$85.98		\$36.28	\$55.82	\$85.89	\$132.15		\$59.86	\$90.01	\$135.34	\$203.51		\$86.92	\$132.15	\$203.51	\$303.76		\$132.15	\$203.51	\$303.76	\$455.52		
0.40x	0.67x	1.10x	1.81x		0.87x	1.39x	2.24x	3.62x		1.29x	2.03x	3.19x	5.03x		2.12x	3.26x	5.02x	7.73x		3.50x	5.26x	7.91x	11.90x		5.26x	7.91x	11.90x	17.85x		7.91x	11.90x	17.85x	27.27x		

NovoCure

People · Opportunity · Context · Deal

Bear \$9.19 Base \$14.00 Bull \$23.58

Underwrite the position the way a venture investor underwrites a round — across all four legs, public or private. **People** is the leg scored here (observable only); Opportunity, Context and Deal cross-reference the rest of the memo.

PEOPLE · WHO RUNS / OWNS / FUNDS IT

Frank Leonard

0 yrs in seat

22.7%
insider ownership

MEDIUM
key-person risk

Capital allocation (realized)
Frank Leonard became CEO on December 1, 2025 — a 15-year NovoCure veteran (prior President/COO/CFO) who replaced Ashley Cordova after her sub-one-year tenure (CEO Jan-Nov 2025), itself following founder-era CEO Asaf Danziger (2002-2024). Capital funds the TTFields oncology-device franchise and new-indication trials (~78% gross margin, net cash); no dividend or buyback. The fresh Phase-3 TRIDENT GBM miss (June 18, 2026 — overall survival 17.7 vs 17.5 months, primary endpoint not met) is the key recent operating event.

Incentive alignment
New CEO Leonard is on a CHF 750,000 base with a 90%-of-base target bonus and standard change-of-control terms. The dominant ownership figure is Executive Chairman William Doyle's ~22.7% beneficial stake (largely via WFD Ventures Fund II, over which he holds sole voting power) — concentration at the non-independent chair, not the CEO. Recent insider Form 4 activity has been routine selling.

GOVERNANCE FLAGS

- single class of ordinary shares, one vote per share (Jersey-incorporated); no controlling shareholder — largest holder Soleus ~9.1%
- non-independent Executive Chairman (Doyle, since 2016) holding ~22.7% beneficially via WFD Ventures Fund II — concentration at the chair
- CEO instability: two transitions in ~13 months (Danziger → Cordova Jan 2025 → Leonard Dec 2025)
- 8 of 11 directors independent (~73%), board declassified (annual election), with a separate lead independent director (Madden, since Feb 2026)
- founder Yoram Palti is off the board since 2018 (now CTO/consultant); his consulting fee is no longer a disclosed related-party item

PEOPLE READ

Clean single-class one-vote shares, a ~73%-independent declassified board and a separate lead independent director are the positives. Holding the score at 3 are a non-independent Executive Chairman with a ~22.7% concentrated stake and back-to-back CEO turnover (two changes in ~13 months) — though the new CEO is a 15-year insider and the device franchise is institutionalized, so key-person risk is moderate. The June 2026 TRIDENT miss is an operating, not a governance, event.

THE FOUR LEGS

People

observable composite · 1–5

Opportunity

The scenario distribution · the business snapshot · the Arthur Indicator

Context

The 7 Powers analysis (Power Audit)

Deal

Open-market common — entry price weighed against the probability-weighted fair value (the +4.5% finding) + position sizing.



NovoCure

supporting analysis · disclaimers · glossary

Bear \$9.19 Base \$14.00 Bull \$23.58

PUSHBACK · WHY THE BASE CASE IS TOO HARSH

- 1

Cheap with a bounded floor
At ~2.5x sales, net cash, 78% GM — the downside is capped, no insolvency risk even in the bear.
- 2

Multiple shots on goal
GBM, lung, pancreatic, brain-mets are four approved/filed indications; one real ramp re-rates the thesis.
- 3

First pancreatic therapy in ~30 years
PANOVA-3 made Optune Pax the first new locally-advanced pancreatic therapy in ~30 years — a differentiated position.
- 4

Unproven operating leverage
The bull needs the first durable operating profit in company history; a decade of losses and SBC over \$100M a year mean that leverage is asserted, not demonstrated.
- 5

Adoption lags labels
Continuous-wear compliance keeps real prescriptions below approved populations — Optune Lua is still only ~\$3M a quarter despite full FDA approval.

FALSIFICATION TRIGGERS

Bull validation Active patients accelerating above ~4,800; Lua/Pax revenue ramping to tens of millions; owner-FCF turning durably positive.	Bear validation Active-patient growth flattening; Lua/Pax stuck in single-digit millions; adj-EBITDA staying negative; reimbursement gating.	Reframe needed A second pivotal trial missing (after TRIDENT) in a new indication, or GBM revenue declining — collapses the multi-indication TAM-expansion thesis.
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Forward-looking statements. Scenarios, valuations, projections, and any forward-looking statements involve substantial assumptions and uncertainty. Actual results may differ materially from those projected. Past performance is not indicative of future results. The model is a model; reality is not.	Author may hold positions. The author may own, intend to acquire, or hold short exposure to \$NVCR or related securities at any time. Positions may change without notice and without an update to this document. Do not assume the author's positions match the tone of the analysis.	Do your own research. Do not make investment decisions on the basis of this document. Consult a licensed financial professional and read the company's official SEC filings (10-K, 10-Q, 8-K, proxy) before forming any investment view. If you wouldn't trust your retirement to a chatbot, don't trust this either.

GLOSSARY · CONCEPTS REFERENCED IN THE NARRATIVE

Tumor Treating Fields (TTFields) — Low-intensity alternating electric fields, via a wearable device, that disrupt cancer-cell division.	Optune Gio / Lua / Pax — The approved products: Gio (glioblastoma, the base), Lua (metastatic NSCLC, early ramp), Pax (locally-advanced pancreatic, newly approved).	Active patients — The key device KPI — patients currently on therapy (~4,791 in Q1 2026); revenue scales with this count, not with label approvals alone.
TRIDENT / PANOVA-3 — Phase 3 trials — TRIDENT (newly-diagnosed GBM + radiation) MISSED its endpoint June 2026; PANOVA-3 (pancreatic) succeeded and enabled Optune Pax.	adj-EBITDA vs owner-FCF — Adjusted EBITDA (-\$34M FY25) excludes stock comp; owner-FCF subtracts the ~\$100M+/yr of SBC, so true owner cash starts well below the headline.	SBC — Stock-based compensation — a real, dilutive cost (a \$43M charge hit Q1'26 on the Optune Pax approval) that the adj-EBITDA figure adds back.